INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Talazoparib · breast monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

**Treatment pattern**
**Ongoing**
Follow the prescribed daily plan

**Regimen source**
**NCCP 00605**
Version 2

Daily continuous treatment

## Inside your handbook


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**My name** _____

**Hospital / oncology team** _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

# Understanding your treatment

Talazoparib · breast monotherapy · National Cancer Control Programme (NCCP) 00605

## How this medicine supports your treatment

Control advanced cancer and symptoms while balancing side effects and treatment burden.

ORAL PARP INHIBITOR

### Talazoparib

**Disrupts deoxyribonucleic acid (the cell's genetic material) (DNA) repair in BRCA-associated cancer**

PARP inhibited / trapped → DNA damage accumulates → BRCA-deficient cells less able to repair

Talazoparib inhibits PARP and traps it on damaged DNA. Cancer cells with inherited BRCA1/2 changes may have less ability to repair this damage. Normal bone marrow can also be affected, so blood-count monitoring is essential.

Report fever, infections, unusual bruising, persistent fatigue or breathlessness.

## Why this treatment?

Talazoparib targets a DNA-repair vulnerability associated with inherited BRCA1/2 changes. Suitability also depends on HER2 status, previous anthracycline/taxane treatment and, for hormone-receptor-positive disease, previous endocrine treatment or its suitability.

### Your treatment setting

Germline BRCA1/2-mutated, HER2-negative locally advanced or metastatic breast cancer

NCCP 00605 v2 · official regimen

### Your exact schedule

Talazoparib is taken once daily continuously until disease progression or unacceptable toxicity. The usual starting dose is 1 mg; kidney function, interactions and side effects can require a different prescription.

NCCP 00605 v2 · official regimen

### Alternatives

Discuss biomarker-directed, endocrine, chemotherapy and supportive-care options appropriate to this setting, including the option not to proceed.

NCCP 00605 v2 · official regimen

## Why this treatment may be chosen

Discuss your own expected benefit, alternatives and uncertainty with the treating team.

## When to get help

**Emergency: 112 / 999.** Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

**Contact your treating team promptly:** Contact oncology urgently for fever, shivers, infection, unusual bleeding, marked fatigue or new breathlessness.

YOUR ONCOLOGY TEAM · DAYTIME

**Insert numbers here**

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

**Insert numbers here**

Ask for medical oncology

EVENINGS / OVERNIGHT

**Your agreed out-of-hours pathway**

Other hospitals: use the contacts your own team provides.

SACTCHECK · CHAPTER 02

# Your schedule, tests and appointments

Talazoparib · breast monotherapy · NCCP 00605

BASELINE → TREATMENT → FOLLOW-UP

## When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00605 v2 · official regimen · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

## Talazoparib · breast monotherapy: treatment and checks

**Your treatment pattern**

Daily continuous treatment

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | First treatment | During treatment | Change / completion | Follow-up |
|---|---|---|---|---|---|
| **Treatment plan** NCCP 00605 | gBRCA / HER2– eligibility | Daily capsules start | Daily without planned breaks | Until progression / intolerance | Next treatment plan |
| **Blood / organ function** NCCP 00605 | full blood count (FBC), renal, liver | Early toxicity review | FBC / renal / liver every 4 weeks | Counts and restart review | Investigate prolonged cytopenia |
| **Symptoms / medicines** NCCP 00605 | Baseline assessment | Early symptom review | Keep the individual check plan | Reconcile at treatment changes | Report continuing symptoms |
| **Cancer review** METASTATIC · MARKERS | Confirm setting | Agree review plan | Response / symptom review | Next treatment / completion | Agreed surveillance |

### What each check is for

1 **Before treatment**

Before the first dose

Germline BRCA1/2-mutated, HER2-negative locally advanced or metastatic breast cancer. Check FBC, kidney and liver function before starting and every four weeks. More frequent blood review may be needed for low counts; prolonged abnormalities require specialist assessment.

**Ask** / **act:** Confirm eligibility, test results and medicine list.

2 **Other checks**

At the specified review points

Confirm the germline BRCA result and previous treatment. Review pregnancy status and contraception, kidney function and interacting medicines, particularly P-glycoprotein inhibitors.

**Ask** / **act:** Confirm who arranges and reviews each check.

3 **Support and practical plan**

At each treatment or phase change

Blood monitoring, adherence, prompt infection/bleeding review and individually prescribed rescue antiemetics.

**Ask** / **act:** Choose the matching support plan; confirm active medicines.

4 **Response and follow-up**

Agreed review points

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment. Keep the agreed follow-up and report persistent or new symptoms between visits.

**Ask** / **act:** Review symptoms, results and patient priorities.

**At the next review, ask**

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Checks across treatment

Your team arranges tests and reviews the results before treatment decisions.

### Blood and organ checks

Check FBC, kidney and liver function before starting and every four weeks. More frequent blood review may be needed for low counts; prolonged abnormalities require specialist assessment.

NCCP 00605 v2 · official regimen

### Other treatment-specific checks

Confirm the germline BRCA result and previous treatment. Review pregnancy status and contraception, kidney function and interacting medicines, particularly P-glycoprotein inhibitors.

NCCP 00605 v2 · official regimen

### Cancer review and follow-up

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment.

ESMO metastatic breast cancer guideline · 2026 · ASCO · biomarkers in metastatic breast cancer

### Bone health

Bone-health assessment follows endocrine therapy, ovarian suppression, menopause and other fracture risks. It has its own monitoring plan.

NICE NG101 · bone health and follow-up

## The wider treatment plan

Review response, toxicity and priorities to decide whether to continue, change or stop. Supportive and palliative care can accompany anticancer treatment.

### Treatment changes

Do not replace a missed or vomited dose. Take the next dose at the usual time. The team decides whether and at what dose to restart after a hold.

NCCP 00605 v2 · official regimen

### Medicines review

Ask the pharmacist to review P-glycoprotein inhibitors and all new medicines or supplements. Changing kidney function or an interacting medicine can change the prescribed dose.

NCCP 00605 v2 · official regimen

### After treatment

Keep the agreed follow-up and report persistent or new symptoms between visits.

NCCP 00605 v2 · official regimen

Report persistent unexplained fatigue, infections or bruising; rare bone-marrow disorders can require investigation.

## Practical treatment details

### Make treatment fit into daily life

Swallow capsules whole with water, with or without food.

Do not open or dissolve them.

### A daily routine

Choose a consistent daily time.

Arrange supplies before they run out.



### Reviews without routine infusion visits

No port or infusion pump is needed for talazoparib itself.

SACTCHECK · CHAPTER 03

# Symptoms and supportive care

Talazoparib · breast monotherapy · NCCP 00605

### Daily treatment and four-weekly reviews

No dates are entered. Columns show milestones, not equal time intervals; follow your individual prescription.

| Treatment or check | Timing in this view |
|---|---|
| **Before starting** | **Before starting**<br>Eligibility, tests and medicines review |
| **Treatment** | **First weeks → Four-weekly review → Ongoing treatment**<br>Daily capsules without a scheduled rest week |
| **Monitoring / support** | **First weeks → Four-weekly review → Ongoing treatment → Next decision**<br>Keep the treatment-specific test and symptom plan |
| **Next decision** | **Next decision**<br>Confirm continuation or completion |

Follow the individual prescription; dose changes are made by the treating team.

Review blood counts and organ function every four weeks or more often when needed.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

## Practical support

### Blood counts

Keep four-weekly blood reviews and any additional tests. Fever or unusual bleeding needs prompt advice.

### Taking capsules

Use the strength and number on your current prescription. Do not replace a missed or vomited dose.

### Nausea

Use the prescribed rescue plan; call if vomiting affects fluids or tablets.

**Medicines review**

Check every new medicine or supplement with the oncology pharmacist.

Taking tablets / attending treatment

## Questions to ask

**Who arranges the checks and explains the results?**

Agree the due window, booking team, result reviewer and contact route if a test or appointment is missing. This page does not record completed checks.

**Explain the plan back:** Which medicines and doses are prescribed, when are they due, which checks are needed, and what would make you call the team today?

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**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Continuous oral treatment

**Active medicines:** Talazoparib

**Emetogenic risk:** Oral minimal-to-low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

### Additional phase-specific support

- No routine antiemetic premedication is required. Select a PRN antiemetic under the oral Table 5/local plan; review renal function, QT risk and interactions.
- Monitor blood counts and organ function every four weeks; transfusion or growth-factor support is individualised.
- Review capsule strength, adherence and interacting medicines before issuing a new prescription.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00605 · VERSION 2

# Talazoparib · breast monotherapy

Talazoparib is taken once daily continuously until disease progression or unacceptable toxicity. The usual starting dose is 1 mg; kidney function, interactions and side effects can require a different prescription.

Oral PARP inhibitor · selected medicine-specific effects. Full details and sources are in the complete reference.

## A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

**1 Blood / infection**

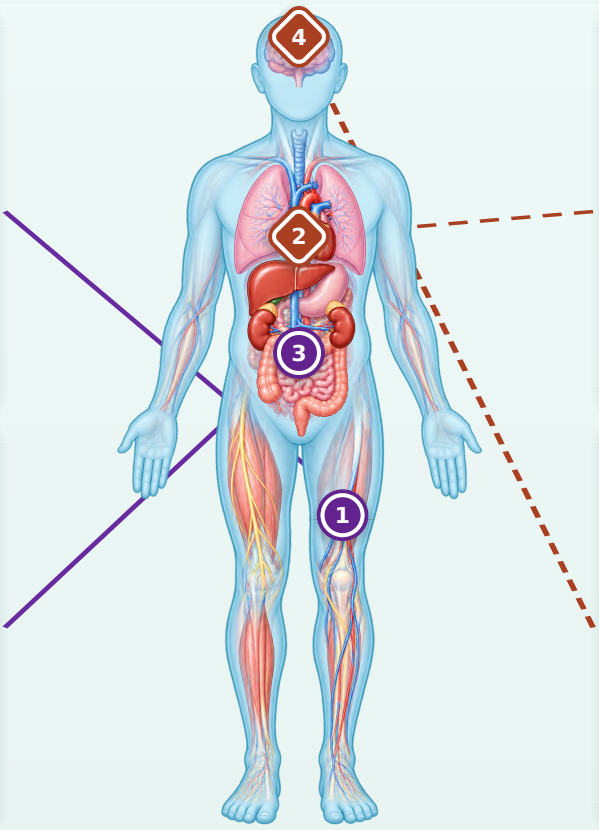
**Talazoparib**

Fever, infection or unusual bruising needs prompt advice.

**2 Energy / breathing**

**Talazoparib**

Marked fatigue or breathlessness may reflect anaemia.




**3 Nausea / appetite**

**Talazoparib**

Call if vomiting or poor intake prevents fluids or medicines.

**4 Hair / scalp**

**Talazoparib**

Hair thinning or loss can occur; ask about practical support.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology urgently for fever, shivers, infection, unusual bleeding, marked fatigue or new breathlessness. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

SACTCHECK · CHAPTER 04

# Your record and contact card

Talazoparib · breast monotherapy · National Cancer Control Programme (NCCP) 00605

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

| Medicine-specific symptom | No concern | Present / manageable | Affects sleep or daily activities — contact team | When / what changed? |
|---|---|---|---|---|
| **Fatigue / breathlessness** | ☐ | ☐ | ☐ | |
| **Infections / bruising** | ☐ | ☐ | ☐ | |
| **Nausea / vomiting** | ☐ | ☐ | ☐ | |
| **Appetite changes** | ☐ | ☐ | ☐ | |

## Checks: expected, booked and reviewed are different

| Test / review | Due window | Who books it? | Who reviews the result? | Outcome / next action |
|---|---|---|---|---|
| _____ | _____ | _____ | _____ | _____ |
| _____ | _____ | _____ | _____ | _____ |
| _____ | _____ | _____ | _____ | _____ |

Missed tablets / injection changes / questions / support needed:

Contact oncology urgently for fever, shivers, infection, unusual bleeding, marked fatigue or new breathlessness. Emergency symptoms: 112 / 999.

# Carry your treatment details

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

## Talazoparib · breast monotherapy

NCCP 00605 v2

Talazoparib is taken once daily continuously until disease progression or unacceptable toxicity. The usual starting dose is 1 mg; kidney function, interactions and side effects can require a different prescription.

Prescriber / next review: _____

**Contact plan**

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
**Emergency: 112 / 999.**

Own hospital / contact: _____

Contact oncology urgently for fever, shivers, infection, unusual bleeding, marked fatigue or new breathlessness.

https://sactcheck.com/patient/00605/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.19 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

SACTCHECK · CHAPTER 05

# Sources and further information

Talazoparib · breast monotherapy · National Cancer Control Programme (NCCP) 00605

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **NCCP 00605 v2 · official regimen**
   https://healthservice.hse.ie/documents/6414/605_v2_Talazoparib_Therapy.pdf
2. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
3. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
4. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
5. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
6. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
7. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
8. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
9. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
10. **eviQ · peripherally inserted central catheter (PICC) patient information**
    https://www.eviq.org.au/p/543
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
12. **NCCP oral antiemetic classification v6 · Tables 5 and 8**
    https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
13. **EMA Talzenna product information**
    https://www.ema.europa.eu/en/documents/product-information/talzenna-epar-product-information_en.pdf